United Nations Commission on Status of Women

Study Guide

Agenda: Addressing Disparities in Access to Assisted Reproductive Technologies

Global School of Science Model United Nations 2026

Table of Contents

History of the Committee

The United Nations Entity for Gender Equality and the Empowerment of Women, also known as UN WOMEN, is the United Nations entity dedicated to advocating and meeting the needs of girls and women around the world. UN WOMEN also promotes the United Nation's 2030 agenda for Sustainable Development Goals and works towards achieving Sustainable Development Goal 15: Gender Equality. The entity abides by articles 1, 2 and the entirety of chapter IV of the United Nations Charter, although its creation was carried out decades later. This entity was founded in response to the United Nations General Assembly resolution 63/311 in July of 2010 and was the result of the merger of the following pre-existing organs of the United Nations: Division for the Advancement of Women (DAW), International Research and Training Institute for the Advancement of Women (INSTRAW), Office of the Special Adviser onGender Issues and Advancement of Women (OSAGI), and United Nations Development Fund for Women (UNIFEM).

UN WOMEN focuses on priority areas that are fundamental to gender equality and meeting the needs of women and girls. The main areas they focus on are: women's leadership and political participation, economic empowerment, ending violence against women, peace and security, humanitarian action, governance and national planning, youth and gender equality, women and girls with disabilities, the 2030 agenda for sustainable development and HIV and AIDS. It also endorses worldwide political efforts to develop internationally agreed-upon gender equality standards and assists UN Member States in implementing those standards by offering expertise and financial assistance. UN WOMEN works through programme initiatives, innovation and technology, intergovernmental support, UN system coordination, and training for gender equality and women's empowerment through research and data.

Introduction to the Topic

Assisted Reproductive Technologies (ARTs) have revolutionised the landscape of reproductive medicine in recent history. Individuals and couples navigating the complexities of infertility and natural conception are largely benefitting from these cutting-edge inventions, which include technologies such as in-vitro fertilisation (IVF), intrauterine insemination (IUI), and egg/sperm donation. Nevertheless, with advances in the field of reproductive health, concerns regarding unequal access to these technologies have also expanded. The question of who is allowed to benefit from such advancements is attached to a wide array of factors, including medical need, socioeconomic, and cultural factors, all systematic barriers that give rise to consequential disparities in their access. The intersectionality of several variables that lead to the unequal allocation of resources for reproductive healthcare is at the core of the issue. Socioeconomic status is a significant obstacle, with those with lower incomes frequently being excluded from access to reproductive treatments. Many people are prevented from exploring these choices due to the exorbitant expenses of ART treatments, which include medication, consultations, and actual operations. The idea that infertility treatments are a luxury rather than an essential component of reproductive healthcare is furthered by this financial barrier. Geographic differences have worsened the matter, as access to specialised reproductive clinics is typically concentrated in metropolitan regions, leaving underdeveloped and rural communities with few alternatives. This distinction between urban and rural areas goes beyond only differences in geographic closeness; it also includes differences in healthcare awareness, infrastructure, and the accessibility of resources for education on reproductive health alternatives. Access to ART is also significantly shaped by cultural variables. The decision of a person or couple to explore assisted reproduction techniques can be greatly influenced by societal standards, religious beliefs, and cultural views regarding the practice. In healthcare settings, stigmatisation and a lack of cultural competency may discourage some people from getting the assistance they require or lead to a lack of information about available alternatives. In addition, differences in access are exacerbated by structural problems such as varied legal frameworks and gaps in insurance coverage. Individuals and couples are severely burdened by the lack of comprehensive insurance coverage for infertility treatments, which puts them in a position where only those with the means can afford to seek ART. Addressing these disparities in access to ARTs is not just a matter of medical ethics; it is a crucial step toward achieving reproductive justice. The rights to parent in secure and encouraging surroundings, to have children, and to not have children are all emphasised by reproductive justice. Ensuring that reproductive healthcare resources are distributed more fairly is essential to protecting these fundamental rights for every person, regardless of their financial status, place of residence, or cultural heritage. As the UN WOMEN committee, we must address the underlying reasons for these differences, their effect on different demographic groups worldwide, and propose solutions to make the environment more equitable for the increasing population of women seeking assisted reproductive options.

History of the Topic

The history of Assisted Reproductive Technologies (ART) is very significant and complex. Due to evolving legal and ethical conundrums, sociocultural perspectives on these practices remain a controversial landscape. The practice of ART can be traced back to 1779, when the Italian priest, Lazzaro Spallanzani made a revolutionary discovery regarding the nature of fertility, establishing that an embryo would be formed through the contact between an egg and a sperm. In that way,the first inseminations were performed, the first being conducted with a dog. Nonetheless, despite subsequent experimentation over the years, artificial insemination with humans was not performed until centuries later. After initial experimentation with artificial insemination, another range of methods of ART began emerging for instance, cryopreservation. The latter method did not come into practice until the mid-1800s when the necessity for sperm banks became apparent as a tool of reproductive technology. Yet cryopreservation gained impetus in 1992 when most veterans decided to freeze their sperm before leaving for the Gulf War. "Semen preparation techniques were developed and intrauterine insemination became popular, being more safe and painless." (Radhey Shyam Sharma et al., 2018) The method of in-vitro fertilisation (IVF) rose dramatically in popularity throughout the 20th century. While the first attempt at IVF can be traced to a rabbit embryo transplantation in 1890, it was not until 1978 that the first successful IVF on a human was performed. Despite the recent developments in ART and its growing acceptance, various obstacles sw3 might hinder its universal appliance.

Nowadays, one could argue religion is one of the greatest barriers to the use of ART worldwide. In fact, according to the World Health Organization, "Representatives of this extreme view are practising Catholics who oppose any of these techniques for the artificiality they imply (even the case of insemination). One of the reasons proffered is the inseparable connection between the unitive and procreative meaning of marriage." (WHO, 2002). According to the World Health Organization in 2002, the three main arguments for religion-based nonacceptance of ART were that it encourages procreation without the involvement of sexual intercourse, that the utilisation of donor gametes is viewed as a third-party involvement in the sanctity of marriage, and that the right to life prohibits the manipulation of embryos and other practices that involve this nature like cryopreservation. Apart from religious implications (which are also present in cultures like Islam, Judaism and Hinduism) ethical and sociocultural barriers have had an important legacy in defining contemporary views of ART. For instance, nowadays a frequent custom in Africa, the Middle East and Asia is the practice of Female Genital Mutilation (FGM). Despite being contested as an utter violation of universal human rights, it comprises a cultural element of gender roles that has been defended due to moral relativism as well. Therefore, it limits the actions that international organisations can have on these communities, adding yet another layer of complexity for ART.

According to NORC in the University of Chicago, "women with FGM/C were more likely to report health concerns related to childbirth, reproductive health, [and] sexual health (...).

Such practice would indeed have created greater disparities and fragmentation upon the acceptance or use of assisted reproductive technologies, as the perpetuation of these cultural traditions passed from generation to generation can be argued to solely distance these ethnic groups from accepting these technologies. This further hinders women's access to further reproductive possibilities, especially in secluded or marginalised communities. The history of ART is further defined by global economic disparities, especially in ensuring access to these technologies for developing nations. Following a research publication by Njagi et al. published in 2023, "Relative ART costs and GDP per capita showed a negative correlation, with the costs in Africa and South-East Asia being on average up to 200% of the GDP per capita." Without adequate government financing mechanisms or the intervention of the international community in assisting with the accessibility to these technologies in less economically developed countries, questions linger on whether ART can truly be an effective tool for reproduction.

Hence, while the history of ART can be construed as shifting and progressing, it still has a long way to go in terms of its universality due to the hindrances that factors such as cultural relativism impose in modern societies.

Current Situation

According to the World Health Organization, in 2023, around 17.5% of the adult population roughly 1 in 6 worldwide, experience infertility (Pan American Health Organization). This shows an evident need to increase access to affordable and high-quality fertility care. Additionally, it is important to bear in mind that infertility does not discriminate, hence the proportion of infertile population worldwide shows a need to widen access to fertility care and ensure that it is no longer sidelined in health research and policies. Despite the magnitude of the issue, assisted reproductive technologies to treat the condition remain underfunded and inaccessible to many due to costs, stigma, and regional availability. One of the most significant barriers to accessing ART is the substantial financial burden associated with fertility treatments. The cost of procedures like in vitro fertilisation (IVF) and fertility medications can be exorbitant, creating a stark divide between those who can afford such interventions and those who cannot. In a monetary sense, in 2018, the average patient in the US paid more than 22,000 USD for a single IVF cycle, a price that does not include the necessary fertility drugs and testing, and the expenses associated with the pregnancy itself ("Single Cycle IVF Cost Details - Advanced Fertility Center of Chicago"). This cost increased to approximately 25,000 USD in a matter of 5 years, by 2023. Albeit, these costs are exclusive to developed countries such as the US and most European countries, given the high-end technologies that have been developed to carry out the procedures. As a result, many opt to travel to countries with cheaper alternatives, yet the travel expenses make the overall costs round up to a similar amount.

Individuals and couples facing infertility often encounter the harsh reality that the ability to pursue fertility treatments is contingent on their financial means. Insurance coverage for ART varies widely, leaving many without the financial support needed to undertake these procedures. The absence of comprehensive insurance coverage exacerbates socioeconomic disparities, as those with lower incomes are disproportionately affected, perpetuating the notion that access to reproductive healthcare is a privilege rather than a basic right. On the other hand, geographic disparities have arisen in recent years as well. Access to specialised fertility clinics is not uniformly distributed, creating a pronounced urban-rural divide. In urban centres, individuals often have more choices regarding fertility clinics and specialised medical professionals, while those in rural or underserved areas may face limited options. The geographical concentration of fertility services in urban locales places an additional burden on individuals from remote or economically disadvantaged regions, who may need to overcome logistical challenges and travel long distances to access reproductive care. The urban-rural gap also extends beyond physical proximity. Rural areas may lack the necessary healthcare infrastructure, including fertility clinics and trained specialists, further limiting access to ART. The consequence is a geographic imbalance in the availability and accessibility of reproductive healthcare resources, amplifying disparities and perpetuating a cycle of unequal access. It is also noteworthy that geographical disparities are based on ethnicity, education level and income. Physical segregation worldwide is still fundamentally determined by discriminatory behaviours among populations, which exemplifies the intersectionality of different factors influencing access to ARTs. Cultural factors also play a pivotal role in shaping individuals' decisions to pursue assisted reproductive interventions, especially in developing and underdeveloped nations. Societal norms and religious beliefs toward fertility treatments can significantly impact choices made by individuals. The stigmatisation of infertility is still widespread, and this in conjunction with certain reproductive technologies within specific cultural contexts may discourage individuals from seeking the required help.

Specifically in regards to the religious aspect of ARTs, procreation is widely encouraged in various religions. For instance, in the Torah (Judaism), followers are instructed to "Be fruitful and multiply, fill the earth and subdue it". In Roman Catholicism, two of the three leading principles related to the family and reproduction are:

(1) the protection of the human being from the moment of its conception,
(2) that the child is the fruit of marriage (God commands husband and wife to have children).

According to the guidelines of Sunni Islam, all forms of assisted reproduction are allowed as long as the sperm and egg are those of the husband and wife and the embryo is placed into the wife's uterus during an existing marriage contract (hence, a third party cannot be involved, which prevents access to treatments including gamete donation or surrogacy). In Shi'a Islam, gamete donation is allowed (Sallam). All of these religious implications are incredibly important to consider particularly in societies where norm and legislation is directly affected by the predominant religion.

Moral concerns driven by cultural factors have also been raised regarding the destruction of human embryos in the practice of in-vitro fertilisation. Embryos are produced with purely reproductive intentions, yet the inevitable destruction of some embryos is an unintended side effect of IVF. In most IVF treatments, once embryos have been produced, it is permissible to destroy them in research, provided that they are unwanted and that the parents' consent is given (Douglas). Nevertheless, the ethical dilemma of the beginning of a "new life" is still widespread, led by religious devotees. Hence, many individuals who would otherwise seek such treatments to deal within-built infertility cannot access them and may refrain from accessing them due to the ethical implications.

In addition to stigma, the lack of cultural competence within healthcare settings can impede effective communication and understanding between healthcare providers and patients. This gap in cultural sensitivity can result in a dearth of information about available reproductive options, further limiting access for individuals from diverse cultural backgrounds. Furthermore, systemic issues, including gaps in insurance coverage and inconsistent legal frameworks, contribute to disparities in access to ART. The absence of a standardised approach to insurance coverage for infertility treatments places a substantial burden on individuals and couples, particularly those without the financial means to self-fund these procedures. Inconsistent legal regulations further complicate the situation, creating policies that vary across jurisdictions and impact access to reproductive technologies globally. Evidently, efforts to address disparities in access to ART require a multifaceted approach that considers the interconnected nature of these challenges. A solution will require a concerted effort from healthcare providers, policymakers, and society as a whole. The complex interplay between socioeconomic, geographic, cultural, and systemic barriers must be recognised by an international body such as the UN WOMEN committee to devise comprehensive strategies to promote reproductive justice. As such, we can ensure that the benefits of ART are accessible to all individuals, regardless of background or circumstance.

Past Actions

Although the history and advancement of assisted reproductive technology is fairly recent, the United Nations has played a significant role in recognising and addressing the disparities as part of its broad commitment to advance global health, human rights, and gender equality. Even though no regional body of the UN directly administers or regulates healthcare services, through conferences and agencies, the UN has served as an advocate towards promoting collaborative efforts to address reproductive health inequalities.

The International Conference on Population and Development (1994)

In 1994, the International Conference on Population and Development (ICPD) in Cairo marked one of the first moments in shaping global discussion on reproductive rights and health. The ICPD emphasised the value of investing in women and girls, affirming the importance of sexual and reproductive health, including family planning, as a precondition for women's empowerment (United Nations Population Fund). The Programme of Action, discussed at the conference, also highlights links between sexual and reproductive health and rights with every aspect of population development, including changes in family structures, urbanisation, and the importance of addressing the rights of young people.

The Millennium Development Goals (2000)

The Millennium Development Goals (MDGs), established in 2000, included targets related to maternal health and gender equality. Goal 5 aimed to improve maternal health, with a specific target to achieve universal access to reproductive health services by 2015 ("United Nations Millennium Development Goals"). While the MDGs did not explicitly mention ART, the broad focus on reproductive health underscored the importance of addressing disparities in access to fertility treatments as part of comprehensive reproductive care.

Sustainable Development Goals (2015)

Building on the MDGs, the Sustainable Development Goals, adopted in 2015, continued to emphasise the importance of universal access to sexual and reproductive health services. Goal 3, which focuses on good health and well-being, includes a specific target to "ensure universal access to sexual and reproductive health-care services, including for family planning, information and education, and the integration of reproductive health into national strategies and programmes" by 2030 (United Nations Department of Economic and Social Affairs). As a result, a more explicit acknowledgement of the role of ART in achieving global health objectives was made.

UNFPA and Reproductive Rights (founded in 1969)

The United Nations Population Fund (UNFPA) has been a key UN agency working to address reproductive health disparities. The UNFPA assists nations in increasing access to a variety of reproductive health services, including infertility treatment. UNFPA supports initiatives to lessen inequities and guarantee that people and couples, regardless of their socioeconomic level or location, have access to complete reproductive healthcare by offering technical assistance, capacity building, and advocacy. Overall, it "works toward the goal of universal access to sexual and reproductive health and rights, including family planning" ("Sexual and Reproductive Health").

Expert Group Meeting on Infertility and Human Rights (2013)

In 2013, the UN Human Rights Council organised an Expert Group Meeting on Infertility and Human Rights. This meeting brought together experts, policymakers, and advocates to discuss the intersections between infertility, human rights, and the right to health. The expert groups addressed key questions about fertility patterns and trends, implications for age structure changes, and other population trends and effective policy responses (United Nations Population Division). The discussed matters were then addressed in the 2014 session of the Commission on Population and Development, which, through a new lens, proposed an assessment of the status of implementation of the Programme of Action of the

ICPD.
Regardless of these efforts, challenges persist. A comprehensive resolution has still not been passed by any major UN Body regarding the endorsement of ARTs globally. Additionally, funding constraints, political differences, and a diversity of cultural and legislative contexts of reproductive healthcare worldwide pose barriers to the implementation of consistent global policies.

Bloc Positions

Africa

Whether it is through their alienation from society or the stigma surrounding the subject, women's reproductive health is largely neglected in African communities. "Except for Egypt and South Africa, until 2001 there has been little or no interest in subfertility prevention and treatment in Africa." (Ombelet and Onofre, 2019)
Given the nature of most African nations, by being classified as less developed countries, both economically and socially, the progress that they have achieved in ART is limited. As aforementioned, Africa's viewpoint of reproductive technology shows ties with their history of gender roles and female genital mutilation (FGM).
Intervention from international entities such as the European Society of Human Reproduction and Embryology and other NGOs, has involved research and evaluation regarding the potential of ART in Africa, concluding on the alarming presence of laboratory costs for medical procedures associated with the manipulation of eggs and embryos.
Pressing economic limitations, complemented with sociopolitical hindrances and taboos largely restrict the expansion of ART in Africa.

America

In Latin America, between 1990 and 2004, there were 150,000 embryo transfer cycles, 33,000 pregnancies and 44,978 babies were born. Nonetheless, the increase in these statistics in Latin America has been inferior to other regions throughout the years.
With the assistance of the REDLARA, an entity that regularises ART clinics and laboratories, Latin America began expanding its use of ART and fertility treatments. Nonetheless, "In no Latin American country is highly complex reproductive assistance covered by public social security plans or prepaid medicine services" (El Hospital, 2021). Hence, economic costs are a major obstacle to the common use of these reproductive technologies.
Governments have formulated financial assistance programs, such as the Uruguayan 'Plan Cigu¨en˜a', which assists patients by offering instalment payments.Nonetheless, this is not a reality for most nations in Latin America. Given the serious involvement of corruption and underdevelopment in Latin American governments, most countries do not possess the financial capabilities to cover such assistance or develop these programmes for their populations.
In the United States, while ART is expensive even with insurance coverage, racial stratification is an important factor that determines the population's access to these

technologies. In fact, according to research published, “increases in ART births generally remain concentrated among White and high-SES women (...)” (Tierney, 2022). Hence, while ART’s economic demand is an inherent challenge, social factors should also be considered hardships in the spread of this technology.

Europe

In Europe, research shows that reported treatment cycle numbers are currently increasing as well as the number of infants born from ART.

In thirteen countries including Austria, Cyprus, Czech Republic, France, Hungary, Ireland, Italy, Lithuania, Malta, Poland, Portugal, Slovakia, and Slovenia, assisted reproductive technology procedures are accessible for medical purposes such as infertility.

Several countries, including Denmark, France, Sweden, the Netherlands, Belgium, Czech Republic, and Slovenia, offer comprehensive public funding schemes for ART. However, in the latter three nations, financial support is contingent upon adherence to specific clinical guidelines, such as the number of embryos transferred relative to the woman's age and the order of the treatment cycle.

In Europe, monitoring and assistance to the population for their access to ART is implemented more successfully, in comparison to other regions, most importantly in the economic sense, as mentioned beforehand.

However, with non-medical treatments, also known as "social" treatments, the narrative is different. Egg freezing, for example: a non-medical treatment, is prohibited in countries like Austria, France, Hungary, Lithuania, Malta, Norway, Serbia, and Slovenia. Conversely, it is permitted in Germany and Switzerland.

Noticeably, there is much division between countries in Europe in their regulations regarding access to ART. In a 2020 press release by the European Society of Human Reproduction and Embryology, “The authors report that "social, cultural or religious" factors in different countries explain some of the variations in regulation and application, especially in third-party donation treatments and surrogacy.” Hence such factors should be considered in the evaluation of Europe’s progress with ART throughout the years.

Asia

Asia has seen a significant change in its landscape with ART throughout recent years. For instance, in 2022, India introduced two significant laws reshaping its fertility sector. These changes respond to concerns over increasing infertility rates and the proliferation of ART clinics in India.

Moreover, nations with low birth rates, such as Taiwan and Japan have taken action regarding ART as well. Taiwan has become a favoured destination in Asia for individuals seeking fertility services such as egg freezing, which is permitted for unmarried women, distinguishing it from several other Asian regions. In July 2021, Taiwan initiated a subsidy program for ART to incentivize local married couples and transnational couples with a Taiwanese ID card, resulting in substantially lower costs for IVF cycles compared to neighbouring countries like Hong Kong, the Philippines, Singapore, and China.

Japan's government has aimed to tackle the social issues generated by its ageing population by providing substantial support for fertility treatments, covering 70% of IVF expenses for up to six cycles, with additional assistance in some rural areas to enhance accessibility.

QARMAs

QARMAs, or Questions All Resolutions Must Answer, are questions that are meant to aid you in the process of creating a solution from the perspective of your country. As the name suggests, these are questions that are mandatory to address in your draft resolution (which can potentially become the final resolution). Hence, we strongly encourage you to consider these questions as you think of your solution as well as the direction you want the committee's discussion to take.

1. How can we address socioeconomic disparities in access to ARTs, ensuring financial constraints do not hinder individuals or couples from availing of the services?

2. In what ways can cultural diversity among member states be considered and seek to promote awareness and understanding of ARTs? Similarly, how can we address potential stigmas and cultural barriers that may discourage individuals from pursuing fertility treatments?

3. What measures can be taken to address geographical barriers, particularly in rural and underserved areas? How can we promote equitable distribution of fertility clinics and specialised medical professionals?

4. How can we navigate legal and ethical challenges associated with implementing ARTs, including concerns related to surrogacy, egg/sperm donations, and other reproductive technologies by taking into account the cultural norms and legal frameworks of member states?

5. What provisions can be made to ensure the inclusion of marginalised and vulnerable populations, such as LGBTQ+ individuals or those facing discrimination based on reproductive health choices, in the accessibility and benefits of assisted reproductive Technologies?

6. How can we promote the need for comprehensive education and awareness programs to inform individuals and communities about their reproductive health options, including ARTs? How can the UN WOMEN committee, under its jurisdiction, support initiatives that promote cultural competence and sensitivity among healthcare

providers in the field of reproductive medicine?

Guidelines for Position Papers

A position paper is a document that outlines a nation's position regarding the topic, as well as possible solutions that address the concerns of the committee. The position paper is divided into 3 main parts which correspond to its paragraphs:

➔ Paragraph 1: Introduction to your country's view on the topic, explain how your country sees the issue being discussed and what should be done, not an introduction to the topic itself.

➔ Paragraph 2: Past actions, explain past actions done by the United Nations and/or your country, and your country's opinions regarding those actions and their Effects.

➔ Paragraph 3:Solutions,offer one or more solutions to the topic,within the scope of what your country can do, while remaining per what your country thinks and has done in the past.

➔ Bibliography: A bibliography formatted according to MLA9 citation guidelines is mandatory. Failure to include a bibliography with any position paper will result in instant disqualification and be considered plagiarism, rendering the delegate ineligible for an award. Furthermore, this document has a specific format. We strongly request delegates to follow the specifications below as any documents that fail to do so will not be accepted.

➔ Font:Times New Roman - 12
➔ LineSpacing: 1.15
➔ Margins: 2.54 cm from all extremities (standard margins)
➔Pages: 1-page max. (excluding bibliography)

Lastly, all position papers are to be delivered by the 12th of April, at 11:59 PM in PDF format to the following email address: (will give you in committee) For ease and to ensure no position papers are lost in spam, please send all of them with the subject "Position Paper - Your Country".

Closing Remarks

Congratulations on getting through this extensive background guide! Bear in mind that the issue being tackled in the committee is complex and layered, and therefore your success in the committee is very dependent on the preparation you do on your country's policies and their interactions with the international community. Your duty as a delegate is to be prepared to meet contrasting ideas from opposing countries with opposing beliefs, which means you must spend a considerable amount of time crafting a solution that can be accepted widely amongst the international community. Having said this, the above study guide is not nearly enough information to tackle this topic, as this guide is simply a brief overview of the matter. As chairs, we will be looking at your ability to embrace your country's position as well as your ability to diplomatically cope with opposing ideals. Your goal in this committee is to drive your country's ideals to their fullest potential by proposing feasible, creative and original solutions that will impress your peers as much as they should impress us. Without further ado, good luck! We are excited to see the ideas that your young, bright minds bring to the table.

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